

Americas Healthcare: ASCO takeaways from the weekend

We are attending the American Society of Clinical Oncology (ASCO) meeting. **SMMT's** partner Akeso presented late-breaking Ph3 HARMONI-6 overall survival data during the plenary session (simultaneously published in The Lancet), where ivonescimab + chemotherapy demonstrated a statistically significant and clinically meaningful 27.9 month benefit vs. 23.7 months for Tevimbra (anti-PD-1) + chemotherapy in frontline squamous non-small cell lung cancer, with a hazard ratio (HR) of 0.66 (one-sided $p=0.0017$), exceeding the ~0.72 bar per our investor conversations and the <0.80 physician bar (see our takeaways here). Additional key presentations from companies within our coverage include: **IMNM** (full Ph3 results for varegacestat in desmoid tumors further support a best-in-class position over Merck KGaA's Ogsiveo, noting all alpha-controlled secondary endpoints including key pain relief metrics achieved statistical significance), **GILD** (updated Ph1 data for GILD's TUB-040 continue to show encouraging results, in-line with the prior update at ESMO, and support a potentially differentiated profile in ovarian cancer; we also note Ph3 subgroup analyses for Trodelvy in 1L TNBC), **INCY** (full Ph3 results for tafasitamab in 1L DLBCL - acknowledging competitive dynamics, we see a potentially differentiated option for certain patients, e.g., GCB-subtype disease), **SMMT** (interim global Ph2 metastatic colorectal cancer data demonstrate durable efficacy at the selected Ph3 ivonescimab dose, including a landmark 9-month PFS rate of 76.1%, supporting the ongoing global Ph3 HARMONI-GI3 study), alongside presentations from Dizal and JNJ with readthrough to **AVBP**.

Summit Therapeutics (SMMT, Buy, Salveen Richter)**Ivonescimab plus chemotherapy versus tislelizumab plus chemotherapy in previously untreated advanced squamous non-small cell lung cancer: overall survival results of the phase 3 harmoni-6 trial**

SMMT's partner Akeso shared overall survival (OS) data from the single-region (China) Ph3 HARMONI-6 study of ivonescimab (ivo; a PD-1xVEGF) + chemotherapy vs. Tevimbra (ONC's anti-PD-1, similar to MRK's Keytruda) + chemotherapy in frontline squamous non-small cell lung cancer (NSCLC) during the plenary session (see our full takeaways from the presentation here). Recall, Akeso has previously shared progression-free survival results from the study (link), where the combination demonstrated a significant and clinically meaningful PFS benefit - 11.1 months vs. 6.9 months -with a hazard ratio (HR) of 0.60 ($p<0.0001$), in patients with squamous NSCLC. Per the late-breaking abstract presented today, ivo + chemotherapy demonstrated a statistically significant overall survival benefit of 27.9 months vs.

Salveen Richter, CFA
+1(212)934-4204 |
salveen.richter@gs.com
Goldman Sachs & Co. LLC

Corinne Johnson
+1(917)343-1445 |
corinne.johnson@gs.com
Goldman Sachs & Co. LLC

Mark Aleynick, Ph.D.
+1(212)357-6820 |
mark.aleynick@gs.com
Goldman Sachs & Co. LLC

Elizabeth Webster, Ph.D.
+1(212)357-9925 |
elizabeth.webster@gs.com
Goldman Sachs & Co. LLC

Matt Dellatorre, Ph.D.
+1(212)855-0830 |
matt.dellatorre@gs.com
Goldman Sachs & Co. LLC

Shrunatra Mishra
+1(332)245-7673 |
shrunatra.mishra@gs.com
Goldman Sachs India SPL

Kevin Strang, Ph.D.
+1(212)357-7227 |
kevin.strang@gs.com
Goldman Sachs & Co. LLC

Erik Wong
+1(212)357-9964 | erik.wong@gs.com
Goldman Sachs & Co. LLC

Tommie Reerink, CFA
+1(212)357-2470 |
tommie.reerink@gs.com
Goldman Sachs & Co. LLC

Lydia Erdman
+1(212)357-6383 |
lydia.erdman@gs.com
Goldman Sachs & Co. LLC

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23.7 months for Tevimbra (anti-PD-1, similar to MRK's Keytruda) + chemotherapy, representing an 34% reduction in the risk of death (HR=0.66; 95% CI, 0.50 to 0.87; one-sided P =0.0017). Ivo + chemotherapy OS benefit was consistent across subgroups, where in PD-L1-negative patients (tumor proportion score (TPS) <1%), median OS was NE versus 18.6 months (HR, 0.64; 95% CI, 0.43 to 0.96), and in patients with PD-L1 TPS≥1%, median OS was NE versus 27.3 months (HR, 0.68; 95% CI, 0.46 to 0.99). On safety, ivo showed and manageable safety profile, which was consistent with prior Ph3 studies of ivonescimab plus chemotherapy. No additional safety signals were noted in the HARMONi-6 study in this current data cut compared to the previous data cut presented.

Overall, the data exceeds the HR<0.72 bar per our diligence and investor conversations, and are impressive in our view, noting this is the first time any regimen has demonstrated a statistically significant benefit over a PD-1 + chemotherapy (the global standard-of-care) in a Ph3 study in frontline NSCLC (see our full takeaways here). Today's HARMONi-6 result further confirms the translation of ivo's PFS benefit to OS, and provides positive readthrough to SMMT's ongoing global Ph3 HARMONi-3 study of ivo + chemotherapy vs. Keytruda + chemotherapy in frontline NSCLC. SMMT will disclose final PFS and interim OS data from the HARMONi-3 study in 2H.

We additionally note a poster presentation from SMMT and Akeso with early Ph1/2 data for ivo in colorectal cancer, with readthrough to the ongoing global Ph3 HARMONi-GI3 study:

Ivonescimab (ivo) with oxaliplatin + fluorouracil (5-FU) + leucovorin calcium (mFOLFOX6) for patients (pts) with unresectable metastatic colorectal cancer (mCRC): A phase 2 study.

The poster contained interim results evaluating ivo at 20 or 10mg/kg + mFOLFOX6 in frontline mCRC. As of the data cutoff (March 31, 2026), 49 patients were enrolled (ivo 20 mg/kg+ mFOLFOX6, n = 25; ivo 10 mg/kg + mFOLFOX6, n = 24), noting 53.1% had KRAS or BRAF tumor mutations, and 67.3% had liver metastases. Per the presenting KOL, the study included a majority of patients from Asia, and 9 patients from the US. Median duration of follow-up was 9.8 months and 9.9 months in the ivo 20 mg/kg + mFOLFOX6 and ivo 10 mg/kg + mFOLFOX6 arms, respectively. ORR was 70.8% in each treatment arm, and all were partial responses. DCR was 100% in each treatment arm, and median PFS was not reached. Treatment responses in the ivo 20 mg/kg + mFOLFOX6 arm were more durable than in the 10 mg/kg + mFOLFOX6 arm, with a duration of response (DoR) estimate at 9 months of 79.1% vs 41.5%, respectively. The landmark PFS rate at 9 months was 76.1% (95% CI, 51.7-89.4) and 70.1% (95% CI, 44.9-85.4) in the ivo 20 mg/kg + mFOLFOX6 and 10 mg/kg + mFOLFOX6 arms, respectively. Treatment related adverse events (TRAEs) occurred in 96% and 87.5% of patients in the ivo 20 mg/kg + mFOLFOX6 and ivo 10 mg/kg + mFOLFOX6 arms, respectively, with grade 3+ events reported in 44% and 33.3%, respectively, with no TRAEs leading to death. The presenting KOL was impressed by the responses, and noted that while a formal regional comparison was not performed, responses were consistent between patients from Asia and the US. The KOL acknowledged a modest increase in toxicity with the higher dose (noting 20mg/kg ivo + mFOLFOX6 is the selected Ph3 HARMONi-GI3 regimen), however highlighted increased efficacy and importantly, a longer DOR with the high dose. Overall, the KOL believes the data supports the ongoing global HARMONi-GI3 study in frontline mCRC.

We highlight notable interest in the Ph2 poster from KOLs during the poster session at ASCO. The data are encouraging in our view, and provide positive read through to SMMT's ongoing Ph3 HARMONi-GI3 study, which is investigating 20mg/kg ivo + mFOLFOX6 in frontline mCRC. Per the presenters, the slight increase in toxicity with the higher Ph3-selected dose, including VEGF-related toxicity, is manageable as the current standard-of-care in frontline mCRC is bevacizumab + chemotherapy, so physicians are accustomed to managing patients with these types of adverse events. Additionally, we note the data appear in-line or better vs. PFE's competing PD-1xVEGF molecule PF'4404, which demonstrated a confirmed ORR of 57.1% and a DCR of 95.2% ([link](#)) in a Ph2 study in 1L mCRC.

Immunome (IMNM, Buy, Salveen Richter)

RINGSIDE: A phase 3 randomized, placebo-controlled trial of varegacestat for treatment of progressing desmoid tumors

IMNM presented full results from the Ph3 RINGSIDE study of gamma-secretase inhibitor varegacestat in desmoid tumors. Recall, IMNM previously shared [positive Ph3 topline data](#) demonstrating a statistically significant progression-free survival (PFS; hazard ratio (HR) = 0.16; $p < 0.0001$) benefit vs. placebo, objective response rate (ORR) of 56%, and tumor volume reduction of 83%, solidifying varegacestat's best-in-class position over Merck KGaA's Ogsiveo. The oral presentation at ASCO this weekend additionally included subgroup analyses and data on key secondary endpoints, including quality of life impacts, where we note all alpha-controlled secondary endpoints demonstrated statistically and clinically significant superiority for varegacestat vs. placebo. On the key endpoint of pain relief, varegacestat demonstrated a statistically significant difference of -2.42 vs. placebo per the Widespread Pain Index (WPI). The presenting KOL noted that while the study was not powered to detect statistical significance between subgroups, varegacestat did show PFS benefit across all subgroups examined, including patients with APC and CTNNBP1 mutations, those with larger tumors, and those who had previously received desmoid tumor therapy.

In the trial discussion portion of the session, the KOL leading the discussion aimed to compare varegacestat against Ogsiveo per the available data from the Ph3 RINGSIDE and [Ph3 DeFi](#) studies, noting deeper responses in patients who received varegacestat when comparing waterfall plots from the two studies, superiority on tumor volume reduction, and that varegacestat appeared to have a better and more rapid improvement in pain relief. On safety, the KOL highlighted varegacestat's superiority on ovarian toxicity (56% of patients with varegacestat, vs 76% for Ogsiveo), albeit noted overall similar levels of other AEs, and a greater number of dose reductions for varegacestat (80%, vs 42% for Ogsiveo in DeFi). The KOL leading the discussion also noted that both drugs appear to show efficacy in intra- and extra-abdominal disease. One remaining question per the KOL is how to define what is the optimal treatment duration balanced against efficacy and toxicity for the GSI class - the KOL highlighted recent [3-year data for Ogsiveo](#), noting that only 3 additional responses were observed with long-term use, thus in the KOL's view the benefit of continuous treatment remains unclear.

Overall, the data further demonstrates varegacestat's best-in-class profile in our view, where we note Varegacestat demonstrated 94.2%/88.9% 1-year/2-year PFS rates, vs. 85%/76% for Ogsiveo in the Ph3 DeFi study, alongside benefits on key patient outcome metrics including pain, per the KOLs. While the presenting KOL noted the lack of a head-to-head study, and

potential similarity given both Ogsiveo and varegacestat are in the GSI class, the KOL did highlight varegacestat's once-daily dosing as differentiating. The KOL leading the discussion portion of the session additionally highlighted varegacestat's superiority, cautioning cross-trial comparisons. We believe varegacestat is differentiated per the efficacy data, once-daily dosing vs. twice-daily for Ogsiveo in the context of physician/patient preference for the easier regimen, and on ovarian toxicity, observed in ~55.6% of premenopausal women vs. ~75% for Ogsiveo. An NDA for varegacestat was submitted in 2Q, and IMNM plans to submit a Marketing Authorization Application to the European Medicines Agency for varegacestat by YE26. Given the profile, we anticipate a stronger launch relative to Ogsiveo and model for global peak sales of \$1.6bn in 2037.

Gilead (GILD, Neutral, Salveen Richter)

NAPISTAR 1-01: Results of phase 1 dose escalation of monotherapy with TUB-040, a novel NaPi2b-targeting exatecan ADC, in patients (pts) with platinum-resistant ovarian cancer (PROC)

At the ongoing ASCO meeting, updated data from the Phase I/IIa NAPISTAR 1-01 trial (NCT06303505) for TUB-040 in platinum-resistant ovarian cancer (PROC) were presented. TUB-040 is a novel antibody-drug conjugate (ADC) targeting the sodium-dependent phosphate transporter NaPi2b, utilizing a topoisomerase I inhibitor (exatecan) payload with a drug-to-antibody ratio (DAR) of 8.

Background. Recall NaPi2b is overexpressed in high-grade serous ovarian cancer and non-small cell lung cancer (NSCLC). TUB-040 is a DAR8 ADC targeting NaPi2b, comprising a humanized Fc-silenced IgG1 mAb conjugated to exatecan via a cleavable, cysteine-selective, stable, solubility-mediating P5 linker providing excellent stability and biophysical ADC properties. In preclinical studies, TUB-040 induces potent antigen-specific cytotoxicity and demonstrates strong bystander activity.

NAPISTAR 1-01 is a phase 1/2a study in biomarker-unselected pts with PROC and NSCLC. TUB-040 was administered Q3W in dose escalation (range 0.5-5.3 mg/kg) to pts with PROC until progression or unacceptable toxicity. As of Dec 1, 2025, 67 PROC pts received TUB-040 for a median of 10 cycles (1-25), median duration of exposure was 213 days (21-546), and 42 (63%) pts were ongoing. Median age: 62 years (34-81), ECOG PS 0-1, median prior lines: 4 (1-7), prior treatment included bevacizumab (84%), PARPi (76%), and mirvetuximab soravtansine (13%).

Efficacy. The efficacy analysis focused on 46 patients treated at dose levels between 1.67 and 3.3 mg/kg. This population was heavily pre-treated, with a median of 4 prior lines of therapy, including bevacizumab (84%) and PARP inhibitors (76%).

- Confirmed Overall Response Rate (cORR): 61% (including two confirmed complete responses).
- Unconfirmed ORR (uORR): 67%.
- Disease Control Rate (DCR): 96%.
- Median Progression-Free Survival (mPFS): 11 months across the broader 67-patient population (doses 0.5–5.3 mg/kg). This significantly outperforms the historical standard of care mPFS of approximately 5 months in PROC.
- CA-125 Response: 81% of evaluable patients (34/42) showed a biochemical

response.

- **Durability:** 79% of responders maintained their response for over 6 months, and 57% for over 12 months. Median duration of response (mDOR) has not yet been reached.

Safety. The safety profile of TUB-040 appears differentiated from other ADCs in the NaPi2b and topoisomerase inhibitor classes, particularly regarding off-target toxicities.

- **Common Adverse Events:** The most frequent treatment-emergent adverse events (TEAEs) were nausea (78%), fatigue (54%), and neutropenia (43.5%).
- **Hematological Toxicity:** Grade 3 or higher neutropenia occurred in 26% of patients at the 1.67–3.3 mg/kg dose levels. Anemia (13%) and thrombocytopenia (4%) of Grade 3+ were also reported.
- **Differentiated Profile:** Notably, there were no clinically relevant cases of interstitial lung disease (ILD)/pneumonitis (only 6.5% Grade 1, all resolved), ocular toxicity, or peripheral neuropathy. This lack of “class-effect” toxicities suggests a wider therapeutic window and potential for combination therapies.

Overall, we view the data as encouraging and broadly in-line with the previous update at ESMO (note the MTD was previously determined to be 4.4mg/kg), and see the drug as one of the leading emerging therapies in ovarian cancer. Physician feedback at the conference was highly positive. Dose optimization is currently ongoing. GILD has noted the potential for a registrational study initiation in FY27.

ASCENT-04: Analysis of efficacy by biomarker subgroups with sacituzumab govitecan (SG) + pembrolizumab (pembro) vs chemotherapy (chemo) + pembro in participants (pts) with previously untreated PD-L1+ metastatic triple-negative breast cancer (mTNBC)

ASCENT-03: Efficacy by biomarker subgroup with sacituzumab govitecan (SG) vs chemotherapy (chemo) in participants (pts) with previously untreated advanced triple-negative breast cancer (TNBC) who are not candidates for PD-(L)1 inhibitors (PD-[L]1i)

Pivotal data for Trodelvy (sacituzumab govitecan) in the first-line (1L) metastatic triple-negative breast cancer (mTNBC) setting were highlighted, primarily through updated subgroup and exploratory analyses from the Phase 3 ASCENT-03 and ASCENT-04 trials. Recall these presentations follow prior publication of the results in the NEJM.

ASCENT-04: Trodelvy + Keytruda (PD-L1 positive)

The ASCENT-04 trial (KEYNOTE-D19) evaluated Trodelvy in combination with pembrolizumab versus standard-of-care (SOC) chemotherapy plus pembrolizumab in previously untreated patients with PD-L1-positive (CPS $\geq$ 10) mTNBC.

- **Primary Efficacy:** The combination demonstrated a statistically significant improvement in progression-free survival (PFS).
 - Median PFS: 11.2 months vs. 7.8 months for the control arm (HR: 0.65; $p < 0.001$).

- Risk Reduction: A 35% reduction in the risk of disease progression or death.
- **ASCO subgroup highlights:** Data presented at the 2026 meeting demonstrated that the PFS benefit was observed across all Trop-2 expression quartiles (Q1–Q4), albeit stronger in high-expressors:
 - High Trop-2 (Q4): Median PFS reached 16.6 months vs. 9.2 months (HR: 0.57).
 - Low Trop-2 (Q1): Median PFS was 9.3 months vs. 9.0 months (HR: 0.81), indicating benefit even in low-expressing tumors.
- **Safety:** The safety profile was manageable and consistent with the known profiles of both agents, with Grade ≥ 3 neutropenia occurring in 43% of patients.

ASCENT-03: Trodelvy Monotherapy (PD-L1 negative/IO ineligible)

The ASCENT-03 trial focused on 1L mTNBC patients who were not candidates for PD-(L)1 inhibitors (primarily PD-L1 negative or those who progressed after prior immunotherapy in the curative setting).

- **Primary Efficacy:** Trodelvy monotherapy significantly outperformed physician's choice of chemotherapy.
 - **Median PFS:** 9.7 months vs. 6.9 months (HR: 0.62; $p < 0.0001$).
 - **Duration of Response (DOR):** 12.2 months for Trodelvy vs. 7.2 months for chemotherapy.
- **ASCO 2026 Subgroup Highlights:** Benefit was maintained across key biomarkers, including tumor BRCA (tBRCA) status and HER2-low status.
 - **tBRCA Wild-type:** HR 0.70.
 - **HER2-Low:** HR 0.74.
 - **Trop-2 Quartiles:** Benefit was observed across all quartiles, with HRs ranging from 0.54 to 0.84.

Overall, we view the updates as incremental (given the prior publication of results). Recall the NCCN guidelines were updated earlier this year to reflect the data (both Trodelvy mono and Trodelvy + Keytruda are now Category 1 options in IO ineligible PD-L1 positive 1L TNBC patients, respectively). Recall Trodelvy is already the SoC in 2L TNBC. We are monitoring competitive dynamics with AZN's Datroway - approved in May for IO ineligible 1L TNBC patients. We note that Trodelvy OS data is still immature, but Datroway achieved a ~5-month benefit (~24 vs. ~19 months; HR = 0.70). We expect clinicians to likely choose between these agents based on toxicity profiles (neutropenia/diarrhea vs. stomatitis/ILD) and the specific PD-L1 status of the patient.

Incyte (INCY, Neutral, Salveen Richter)

frontMIND: Phase 3 study of tafasitamab (Tafa) plus lenalidomide (Len) and R-CHOP for patients (pts) with newly diagnosed diffuse large B-cell lymphoma (DLBCL).

Background. Recall tafasitamab is an Fc-engineered monoclonal antibody targeting CD19 on malignant B cells, with enhanced affinity to immune effector cells that drives antibody-dependent cellular cytotoxicity (ADCC), antibody-dependent cell-mediated

phagocytosis (ADCP), and direct cytotoxicity. Lenalidomide synergizes with tafasitamab by expanding and activating effector cells and enhancing these mechanisms. Despite first-line R-CHOP being the standard of care for diffuse large B-cell lymphoma (DLBCL), ~40% of high-risk patients are not cured.

frontMIND (NCT04824092) is a Ph3, global, multicenter, placebo-controlled, double-blind, randomized study evaluating Tafa-Len-R-CHOP vs R-CHOP in previously untreated patients aged 18–80 years with high intermediate- or high-risk (IPI 3–5, aaIPI 2–3 if ≤60 years) DLBCL or high-grade B-cell lymphoma (HGBL). A total of 899 patients were randomized 1:1 to receive 6 cycles of either Tafa-Len-R-CHOP or R-CHOP plus placebos. The primary endpoint was progression-free survival (PFS), with key secondary endpoints of event-free survival (EFS) and overall survival (OS).

Efficacy

- **PFS (Primary endpoint):** Tafa-Len-R-CHOP significantly prolonged PFS vs R-CHOP, with a hazard ratio (HR) of 0.75 (95% CI: 0.59–0.96; P=0.0194), representing a 25% reduction in risk of progression or death. The 2-year PFS rate was 71.1% vs 62.9% (Δ =8.2%), and the 3-year PFS rate was 67.3% vs 60.7% (Δ =6.6%).
 - EFS: Significantly improved with Tafa-Len-R-CHOP (HR 0.79; 95% CI: 0.64–0.97; P=0.0260); 2-year EFS was 65.0% vs 56.7%.
 - Centrally Confirmed Subtypes (n=773): PFS benefit was more pronounced (HR 0.68; 95% CI: 0.52–0.88), with a 2-year PFS difference of 10.5%.
 - Subgroup Analyses: Consistent trends toward PFS benefit were observed across prespecified subgroups, including both ABC-like and GCB-like cell-of-origin subtypes, IPI risk groups, geographic regions, and age groups.
- **OS (Interim):** A positive trend was observed (HR 0.85; 95% CI: 0.63–1.14; P=0.2703); 2-year OS was 84.1% vs 80.5%.
- **Response rates:** ORR (80.4% vs 76.1%) and PET-negative CR rates (both 65.2%) were similar between arms, with a numerically higher partial response rate in the Tafa-Len-R-CHOP arm.

Safety

- Treatment-emergent adverse events (TEAEs) were reported in nearly all patients in both arms (98.6% vs 97.1%).
- Grade ≥3 TEAEs occurred more frequently with Tafa-Len-R-CHOP (86.7% vs 76.1%), as did serious TEAEs (50.1% vs 38.9%) and fatal TEAEs (5.9% vs 3.8%).
- The most common grade ≥3 events were cytopenias (neutropenia, anemia, thrombocytopenia), with serious febrile neutropenia occurring in 13.3% vs 9.8%.
- Fatal TEAEs were largely balanced, except for higher rates of COVID-19 and sepsis in the experimental arm.
- Discontinuation rates due to TEAEs were similar (5.2% vs 5.4%).
- Importantly, the addition of tafasitamab and lenalidomide did **not** compromise delivery of the R-CHOP backbone, with median relative dose intensities of 100% for most components across the 6 cycles.

Key Conclusions

1. Tafa-Len-R-CHOP significantly prolonged PFS compared with R-CHOP (HR 0.75) in patients with previously untreated, high-risk DLBCL and HGBL.
2. Consistent trends toward PFS advantage were observed across key prespecified subgroups.
3. The incremental safety events with Tafa-Len-R-CHOP were well-managed and did not impact delivery of the R-CHOP backbone.
4. These results support **Tafa-Len-R-CHOP as a new standard first-line treatment option** for patients with high-risk DLBCL or HGBL, regardless of cell-of-origin molecular subtype. The full manuscript has been published in *The Lancet*.

Overall, we view the data as encouraging (following the prior topline announcement), and INCY plans to submit regulatory applications later this year. While we note limited attendance/KOL focus at the conference, the key debate was around how Tafa-Len-R-CHOP would compete with Pola-R-CHP as the preferred front-line option for high-risk DLBCL. Recall, Pola-R-CHP is a modification of the standard R-CHOP backbone - it substitutes vincristine with polatuzumab vedotin, an antibody-drug conjugate (ADC) and is currently considered a standard-of-care option alongside R-CHOP and is the primary benchmark for newer combinations like the Tafa-Len-R-CHOP regimen. KOLs were optimistic regarding Tafa-Len-R-CHOP's efficacy across molecular subtypes.

ArriVent Biopharma (AVBP, Buy, Corinne Johnson)

We highlight read-through to AVBP from two ASCO 2026 presentations: (1) Dizal's Ph3 WU-KONG28 data for sunvozertinib in 1L EGFR exon20ins NSCLC, and **(2)** JNJ's updated CHRYSALIS-2 Cohort C data for Rybrevant + Lazcluze in 1L atypical EGFR NSCLC.

- At ASCO 2026, Dizal presented Ph3 WU-KONG28 data showing sunvozertinib (sunvo) monotherapy delivered 10.3mo mPFS vs. 7.5mo chemo, with a 68.1% ORR in 1L EGFR exon20ins. We view this as incrementally positive AVBP's firmonertinib (firmo) ahead of Ph3 Furvent, as the 7.5mo chemo control (reinforcing our ~8mo FURVENT assumption) and 10.3mo drug mPFS provides a helpful benchmark that doesn't set too high a bar on efficacy or safety vs. our expectations for firmonertinib. Our recent Furvent statistical analysis suggests firmo delivers mPFS >11mo (~16mo base case, reliant on an assume ~8m control arm outcome), which screens better than sunvo and competitive with/better than Rybrevant's 11.4mo mPFS. Additionally, sunvo and firmo share a similar MOA - oral, irreversible, mutant-selective EGFR TKIs targeting exon 20 insertions - so the positive Ph3 outcome further de-risks the mechanism for firmo.
- JNJ reported updated CHRYSALIS-2 Cohort C data for Rybrevant + Lazcluze in 1L atypical EGFR NSCLC, showing a 57% ORR and ~41mo mOS. We view this as incrementally positive for AVBP because: (1) on a non-randomized cross-trial basis, firmo appears numerically favorable, which delivered 82%/68% ORR/cORR (n=22) in comparable atypical EGFR patients (Ph1b PACC study); (2) JNJ's study enrolled a relatively more drug-sensitive atypical EGFR population, centered on classical atypical mutations such as G719X, L861Q, and S768I, and excluded exon 20

insertions, whereas firmo development targets the harder-to-treat PACC mutations and exon 20 insertions, in our view; and (3) the mature ~3.5-year OS benchmark continues to validate atypical EGFR as a meaningful, durable commercial target, de-risking the broader market opportunity.

Valuation and Risks

AVBP (Buy). Our 12-month price target of \$34 is based on a DCF analysis that assumes a 15% discount rate and 2% terminal growth rate; these assumptions reflect AVBP's stage of clinical development and modest long-term expansion opportunity. Key downside risks include: (1) negative clinical outcomes for firmonertinib in pivotal trials for lead EGFR ex20ins / PACC mutation / classical mutation programs, (2) worst-than-anticipated competitive dynamics from established therapies, (3) regulatory delays and/or unexpected changes in FDA criteria for approval, and (4) financial risk due to cash needs prior to turning profitable.

GILD (Neutral): Our 12-month price target of \$130 is based on a 100% DCF analysis (3% TGR, 8% WACC). Downside risks: Clinical or regulatory failures or delays (in particular, for the long-acting HIV portfolio); Headwinds/disruptions to HIV market; Weaker than expected Biktarvy or Yeztugo sales growth; Success of competitor drugs. Upside risks: Market growth and/or penetration greater than our assumptions; Less pricing pressure from generics in HIV market in 2H of decade; Clinical or regulatory failures or delays of competitor drugs.

INCY (Neutral): Our 12-month price target of \$100 is based on 100% DCF (9% WACC and 1% TGR). Risks: Upside risks: Faster-than-anticipated progress across Jakafi lifecycle management programs including positive data for mCALR, better-than-expected launch for SNDX-partnered Niktivmo, or greater uptake for Opzelura and approval in expansion opportunities. Downside risks: Failure or safety events for any of the underlying clinical programs given the emphasis on the pipeline development, regulatory delays, and the potential threat from competing oncology/GVHD programs. Headwinds to Opzelura.

SMMT (Buy). Valuation: Our 12-month PT of \$41 s based on a blended 85%/15% risk-adjusted DCF value of \$35 (WACC of 18% and TGR of 2%) and a theoretical M&A value of \$73 (11x 2031E sales). Downside risks: Clinical, technology and manufacturing risk - data generated by partner Akeso in China may not translate to SMMT's global clinical studies; U.S.-China trade relations may adversely impact supply chain operations; unexpected safety issues or clinical delays may push launch timelines. Regulatory risk - failure to secure regulatory approval for indications or territories could impact commercial success and would pose significant downside risk to our sales estimates. Competitive risk - competitor PD-1/L1xVEGF assets could demonstrate superior data, or advance through clinical development ahead of expectations. Financing risk/dilution.

IMNM (Buy). Valuation: Our 12-month PT of \$35 is based on a blended 85%/15% risk-adjusted DCF value of \$27 (WACC of 17% and TGR of 5%) and a theoretical M&A component of \$83 (11x 2031E sales following 2 full years of IM-1021 sales). Downside risk: Clinical/technology risk: If IMNM's targeted oncology programs fail to demonstrate sustained and durable benefit, their long-term potential may be inhibited with limited physician uptake, and unexpected safety issues or clinical delays may push launch

timelines. Competitive risk: competitor assets could advance through clinical development ahead of expectations or generate data that appears superior to IMNM's assets. Regulatory risk: inability to secure regulatory approval for indications or territories, regulatory requirements and timelines may affect trial scope and timelines. Commercial risk: despite receiving marketing approval, IMNM's product may fail to achieve commercial success given potential manufacturing difficulties as a result of production scale up. Financing/dilution risk, and lower-than-expected sales would also provide downside risk to our estimates.

Disclosure Appendix

Reg AC

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Contributing Authors: Salveen Richter, CFA Goldman Sachs & Co. LLC, Corinne Johnson Goldman Sachs & Co. LLC, Mark Aleynick, Ph.D. Goldman Sachs & Co. LLC, Elizabeth Webster, Ph.D. Goldman Sachs & Co. LLC, Matt Dellatorre, Ph.D. Goldman Sachs & Co. LLC, Shrunatra Mishra Goldman Sachs India SPL, Kevin Strang, Ph.D. Goldman Sachs & Co. LLC, Erik Wong Goldman Sachs & Co. LLC, Tommie Reerink, CFA Goldman Sachs & Co. LLC, Lydia Erdman Goldman Sachs & Co. LLC.

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ArriVent Biopharma Inc. (Buy, \$30.27), Gilead Sciences Inc. (Neutral, \$134.43), Immunome Inc. (Buy, \$21.83), Incyte Corp. (Neutral, \$96.74) and Summit Therapeutics Inc. (Buy, \$17.54)

The rating(s) for Gilead Sciences Inc., Immunome Inc., Incyte Corp., Regeneron Pharmaceuticals Inc. and Summit Therapeutics Inc. is/are relative to the other companies in its/their coverage universe: 4D Molecular Therapeutics, Acadia Pharmaceuticals Inc., Agios Pharmaceuticals Inc., Alnylam Pharmaceuticals Inc., Amgen Inc., Annexon Biosciences, Apogee Therapeutics Inc., BioMarin Pharmaceutical Inc., Biogen Inc., CRISPR Therapeutics, Denali Therapeutics Inc., Enliven Therapeutics Inc., Generate Biomedicines Inc., Gilead Sciences Inc., Immunome Inc., Incyte Corp., Intellia Therapeutics, Ionis Pharmaceuticals Inc., Moderna Inc., Rapport Therapeutics, Regeneron Pharmaceuticals Inc., Relay Therapeutics, Sarepta Therapeutics Inc., Summit Therapeutics Inc., Taysa Gene Therapies Inc., Ultragenyx Pharmaceutical, Vertex Pharmaceuticals Inc., uniQure

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	Rating Distribution				Investment Banking Relationships		
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Target price history table(s)

Incyte Corp. (INCY)

Date of report	Target price (\$)	Closing price (\$)
17-Apr-26	100.00	97.82
11-Feb-26	98.00	98.84
08-Jan-26	90.00	105.92
29-Oct-25	80.00	90.18
29-Jul-25	75.00	77.38
29-Oct-24	70.00	73.60
04-Oct-24	63.00	67.49
30-Jul-24	60.00	67.79
16-Jul-24	61.00	64.00
30-Apr-24	60.00	52.05
14-Feb-24	63.00	57.89
21-Nov-23	65.00	53.23
01-Nov-23	98.00	54.37
01-Aug-23	114.00	63.69

Gilead Sciences Inc. (GILD)

Date of report	Target price (\$)	Closing price (\$)
17-Apr-26	130.00	137.64
11-Feb-26	125.00	155.80
08-Jan-26	115.00	120.67
31-Oct-25	110.00	119.79
17-Oct-25	108.00	122.81
08-Aug-25	105.00	119.41
25-Apr-25	100.00	103.17
14-Apr-25	97.00	106.50
12-Feb-25	96.00	103.31
27-Jan-25	85.00	95.35
07-Nov-24	84.00	97.90
04-Oct-24	74.00	84.59
16-Jul-24	71.00	71.64
15-Apr-24	73.00	67.75
07-Feb-24	76.00	74.42
22-Jan-24	82.00	78.43
08-Nov-23	84.00	77.90
04-Aug-23	85.00	78.68

ArriVent Biopharma Inc. (AVBP)

Date of report	Target price (\$)	Closing price (\$)
14-Nov-25	34.00	20.69
10-Jul-25	33.00	22.03
14-Nov-24	40.00	28.08
10-Sep-24	38.00	27.29
01-Apr-24	28.00	18.31
20-Feb-24	27.00	20.63

Regeneron Pharmaceuticals Inc. (REGN)

Date of report	Target price (\$)	Closing price (\$)
29-Apr-26	968.00	686.36
30-Jan-26	967.00	741.45
08-Jan-26	914.00	801.01
19-Dec-25	807.00	767.96
29-Oct-25	788.00	652.91
04-Aug-25	785.00	571.54
30-Apr-25	804.00	598.76
11-Apr-25	917.00	554.18
05-Feb-25	1,019.00	716.72
27-Jan-25	1,182.00	684.71
31-Oct-24	1,185.00	838.20
04-Oct-24	1,294.00	1,012.82
02-Aug-24	1,292.00	1,082.19
02-May-24	1,180.00	937.61
15-Apr-24	1,126.00	898.56
04-Apr-24	1,121.00	937.02
02-Feb-24	1,083.00	945.18
10-Jan-24	1,007.00	906.16
02-Nov-23	1,060.00	818.79
13-Oct-23	1,064.00	839.63
04-Aug-23	1,028.00	766.44
13-Jul-23	990.00	720.98

Summit Therapeutics Inc. (SMMT)

Date of report	Target price (\$)	Closing price (\$)
02-May-25	41.00	27.90
14-Apr-25	37.00	26.13
28-Feb-25	42.00	20.69

Immunome Inc. (IMNM)

Date of report	Target price (\$)	Closing price (\$)
04-Mar-26	35.00	22.52
22-Sep-25	26.00	9.77

Price targets shown in table(s) are unadjusted for corporate actions.

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